



MOTHERHOOD (EARLY) / MARRIAGE · NEW MOTHER (0-6 MONTHS) + PARTNER

Every Night, Just Me

A postpartum night-coordination system that replaces nightly arguing with a pre-agreed roster, a fairness ledger, and the exact words to make it stick.

A written, fair night-shift system both adults follow — so she gets a protected sleep opportunity, and nobody has to ask.



What This System Is *And Isn't*

PRODUCT NAME	Every Night, Just Me
SUBTITLE	A postpartum night-coordination system that replaces nightly arguing with a pre-agreed roster, a fairness ledger, and the exact words to make it stick.
ONE-LINE PROMISE	A written, fair night-shift system both adults follow — so she gets a protected sleep opportunity, and nobody has to ask.
LIFE AREA	Motherhood (early) / Marriage
LIFE STATE	New Mother (0–6 months) + partner
TARGET PERSON	A breastfeeding or formula-feeding mother, 0–12 weeks postpartum, doing 100% of night care while her partner sleeps through or is minimally involved
SITUATION	She is running every night alone — feeding, settling, soothing — while her partner sleeps, works away, or believes "helping" means one diaper change. There is no agreed system; every night is renegotiated by exhaustion and resentment.
PROBLEM	She cannot get a single protected sleep block, is approaching or past the point of physical/cognitive breakdown, and the marriage is absorbing the damage in silence
CONSTRAINTS	She is often still breastfeeding (so "sleep through the night" isn't simply available to her), cultural norms assign night-care to the mother by default, asking directly for help can feel like an accusation, and — for some buyers — there is no partner in the home at all
DESIRED TRANSFORMATION	A written, fair night-shift system both partners follow, designed to protect a 4–5 hour sleep opportunity for the mother whenever circumstances allow, without the nightly argument about whose turn it is

Thirteen Modules. *One System.*

01

The Read — The Argument That Ends the War

When you are waking up every 90 minutes to a crying baby while your partner sleeps peacefully in the next room, the issue feels deeply personal. It feels like a lack of love, a lack of awareness, or a lack of care.

READ

02

The Situation Finder (Self-Triage Quiz)

Before choosing a schedule, we must diagnose your current night reality. Answer the three questions below honestly. Your answers route you to the exact roster and scripts built for your situation.

DECIDE

03

The Decision Tree

Use your Situation Finder answers to confirm your route. Follow the tree from the top.

DECIDE

04

Roster Safety & Boundary Rules

Before signing any roster, both adults must read and agree to the following guardrails. These are not optional. They are the safety boundary within which the entire system operates.

READ

05

The Three Night-Shift Contract Templates

To protect a minimum of four (4) hours of uninterrupted sleep opportunity for [Mother's Name] every night to prevent cognitive breakdown, while ensuring [Baby's Name] is safely cared for.

BUILD

06

The Sleep Banker (Recovery Bank) Mechanics

The Recovery Bank is a fairness accounting system. It translates invisible night exhaustion into visible, tradable credits — so that exhaustion debt does not silently compound into resentment.

TRACK

07

The Seven Script Cards

These scripts are not suggestions. They are precision tools — written for the specific moment, with the specific words that do not trigger defensiveness. Do not improvise at 3 AM. Use the script.

PRINT

08

The Fridge Shift Chart (Printable Layout)

Print this page. Put it on the refrigerator or any surface both adults see every night. This is the single source of truth for

tonight's roster. No app, no memory, no argument — just the chart.

[PRINT](#)

09

The 14-Day Shift Tracker

Do not just track whether you followed the roster. Track how the system performs. This data feeds your Sunday Roster Review and your Day 30 check-in.

[PRINT](#)

10

The Rescue Card (Bad Night Protocol)

Before anything else — check the baby.

[PRINT](#)

11

The Re-entry Protocol

You had a good run — maybe a week, maybe two. Then something happened: a work trip, an illness, a hard season, an argument. The roster quietly stopped. You slid back to the pattern where you do everything alone at night,

[RECOVER](#)

12

Safety Gate — When Exhaustion Needs More Than a Night Plan

This night-shift system is designed to protect your sleep opportunity and reduce household conflict. It does that well.

[ESCALATE](#)

The *Night-Shift Roadmap*

1

READ — The Read — The Argument That Ends the War

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Before signing any roster, both adults must read and agree to the following guardrails. These are not optional. They are the safety boundary within which the entire system operates.

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BUILD — The Three Night-Shift Contract Templates

To protect a minimum of four (4) hours of uninterrupted sleep opportunity for [Mother's Name] every night to prevent cognitive breakdown, while ensuring [Baby's Name] is safely cared for.

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TRACK — The Sleep Banker (Recovery Bank) Mechanics

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PRINT — The Seven Script Cards

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PRINT — The Fridge Shift Chart (Printable Layout)

Print this page. Put it on the refrigerator or any surface both adults see every night. This is the single source of truth for tonight's roster. No app, no memory, no argument — just the chart.



PRINT — The Rescue Card (Bad Night Protocol)

Before anything else — check the baby.



RECOVER — The Re-entry Protocol

You had a good run — maybe a week, maybe two. Then something happened: a work trip, an illness, a hard season, an argument. The roster quietly stopped. You slid back to the pattern where you do everything alone at night,

Four Pages You Should *Print Tonight*

07

The Seven Script Cards

These scripts are not suggestions. They are precision tools — written for the specific moment, with the specific words that do not trigger defensiveness. Do not improvise at 3 AM. Use the script.

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The 14-Day Shift Tracker

Do not just track whether you followed the roster. Track how the system performs. This data feeds your Sunday Roster Review and your Day 30 check-in.

10

The Rescue Card (Bad Night Protocol)

Before anything else — check the baby.

The Read — The Argument That Ends the War

When you are waking up every 90 minutes to a crying baby while your partner sleeps peacefully in the next room, the issue feels deeply personal. It feels like a lack of love, a lack of awareness, or a lack of care.

The next morning, the conversation usually goes like this:

Mother: I need you to help more at night. I am dying here.

Partner: Just tell me what to do, I'll help. But you know I have work tomorrow.

That night, the baby cries. The mother lies awake waiting for the partner to wake up. He doesn't. She becomes furious, nudges him aggressively, or simply gets up herself, muttering in resentment.

Here is the hard truth: **Unstructured nightly negotiation fails 100% of the time.**

It fails for three structural reasons:

1. The Burden of Asking

Asking your partner to "help" is actually cognitive labor. You have to wake up, evaluate if the cry is urgent, decide to wake him, and instruct him on what to do. By the time you have done all that, you are already fully awake and your sleep cycle is ruined.

2. The "Helper" Reframe

When a partner is positioned as a "helper," they wait for instructions. They do not take ownership. If the baby is wet, they wait to be told to change the diaper. If the baby won't settle, they hand the baby back because "they want you."

3. Exhaustion Negotiation

At 3:00 AM, both of your brains are operating at near-zero capacity. Arguing about "whose turn it is" under the influence of severe sleep deprivation is like trying to negotiate a contract during a mild concussion.

The Solution: Shift-Based Adult Coordination

We must stop negotiating at night. We must externalize the schedule.

A written contract removes the emotional labor of asking. It converts a nightly confrontation into a pre-agreed default. Nobody has to ask — the schedule asks. The partner ceases to be a "helper" and becomes a **co-manager** who owns a specific, non-optional shift block.

This is not about blame. It is about building a system that works even when you are too exhausted to be generous with each other.

The Situation Finder (Self-Triage Quiz)

Before choosing a schedule, we must diagnose your current night reality. Answer the three questions below honestly. Your answers route you to the exact roster and scripts built for your situation.

1

What is your primary feeding method?

A

I am exclusively breastfeeding (direct latch only, no bottles).

B

I am breastfeeding but can pump expressed milk, or we combo-feed with formula.

C

We feed exclusively with formula or expressed milk.

2

What is your partner's availability at night?

A

My partner is home at night but is minimally involved or sleeps through.

B

My partner works night shifts, travels constantly, or lives in another city.

C

I am parenting solo — I do not have a partner in the home.

3

What is your primary source of household friction right now?

A

My partner says they "want to help" but sleeps through the cries or doesn't know what to do.

B

We constantly argue at night about whose turn it is or who is more tired.

C

Extended family members (mother, mother-in-law, aunts) say night duty is solely the woman's job and criticize my partner's involvement.

D

I am too exhausted to coordinate anything — we are in absolute survival mode.

Your Results

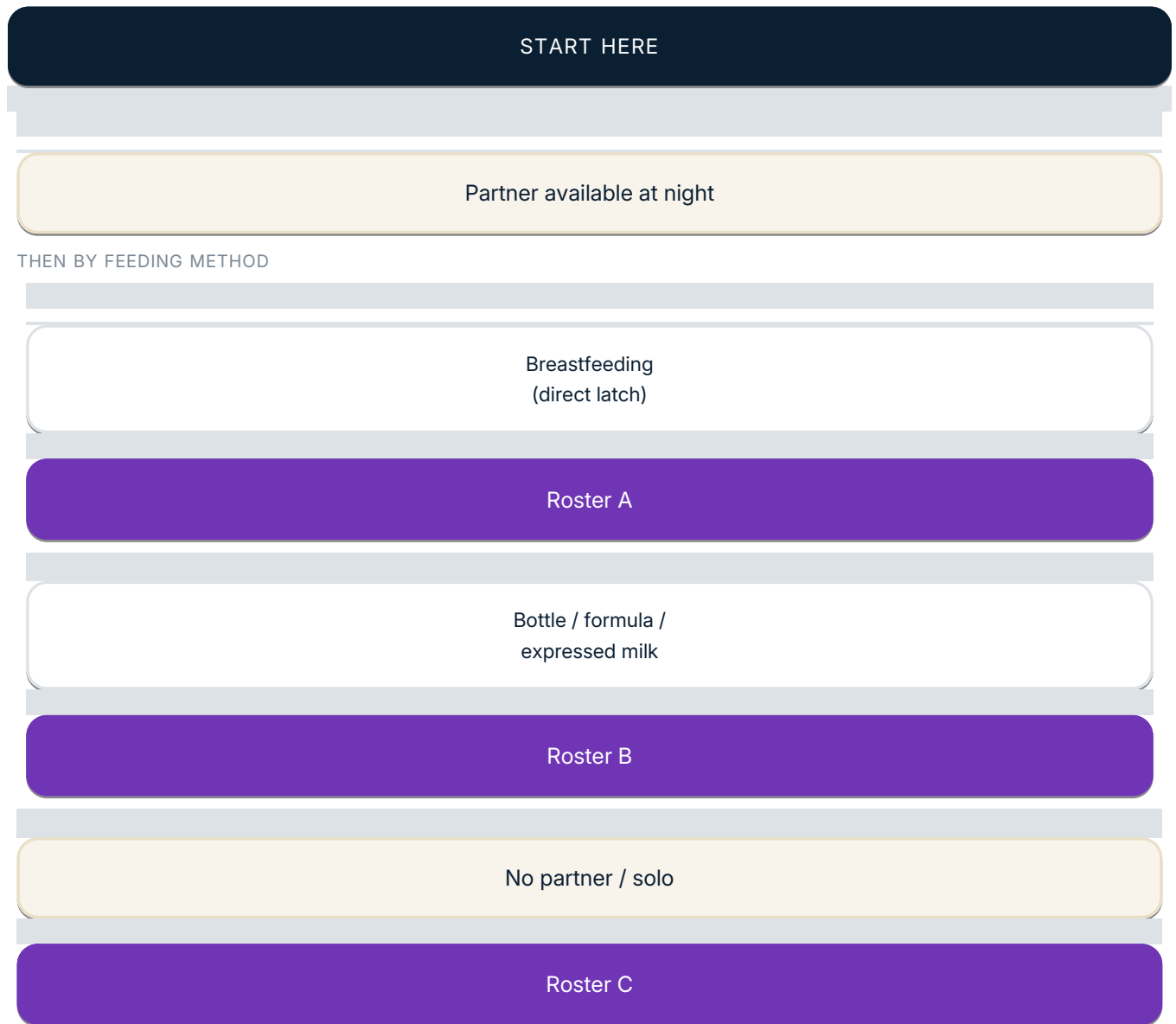
Q1 ANSWER	Q2 ANSWER	Q3 ANSWER	→ YOUR ROUTE
A (breastfeeding only)	A (partner home)	A, B, or C	Roster A Scripts 1, 2, 3
B or C (bottle capable)	A (partner home)	A, B, or C	Roster B Scripts 5, 6
Any	B or C (no partner/solo)	Any	Roster C Script 7
Any	Any	D (survival mode)	Go to Part 11: Rescue Card first, return here tomorrow

• The Situation Finder (Self-Triage Quiz)

Module 02 · DECIDE

The Decision Tree

Use your Situation Finder answers to confirm your route. Follow the tree from the top.



Routed to Roster A

You are breastfeeding and your partner is home. Your partner manages all non-feeding duties during his shift — diapering, soothing, resettling. He brings the baby to you only for feeds, then takes the baby back immediately. You do not get up for anything else. Start with: Part 6 → Roster A Template. Use Script Cards 1, 2, and 3 to introduce and hold the system.

Routed to Roster B

You can use bottles (formula or expressed milk). This allows a true alternating shift — one parent fully off, one parent fully on. Both get a solid 4–5 hour sleep opportunity window. Start with: Part 6 → Roster B Template. Use Script Cards 5 and 6 for weekly review and reset.

Routed to Roster C

You are parenting without a partner at night — solo, or partner is away. This roster builds a scheduled support web from your available village: a family member, a paid night helper, or a trusted friend. Start with: Part 6 → Roster C Template. Use Script Card 7 for managing extended family dynamics.

Survival Mode

Do not start the roster tonight. Go directly to Part 11: The Rescue Card (Bad Night Protocol). Get through tonight. Return to this Decision Tree tomorrow when you have had any rest at all.

Roster Safety & Boundary Rules

Before signing any roster, both adults must read and agree to the following guardrails. These are not optional. They are the safety boundary within which the entire system operates.

RULE 1: INFANT SAFETY IS PARAMOUNT — ALWAYS

SAFE-SLEEP STANDARDS

This contract controls adult coordination only. It does NOT override safe-sleep practices. The baby must always sleep on their back, on a firm, flat, separate sleep surface (crib or bassinet), free of blankets, pillows, bumper pads, or soft toys — in line with World Health Organization (WHO) and pediatric guidelines. If you are unsure what safe sleep looks like for your baby's age and stage, ask your pediatrician or midwife before implementing any roster.

VIGILANCE REQUIREMENT

The active Shift Manager must remain fully awake and alert during their shift while handling the baby. Never hold the baby in a bed, sofa, or armchair if there is any risk of falling asleep. If you feel unsafe from fatigue, activate the emergency handoff immediately (see Part 11).

NO EARPLUGS FOR THE ACTIVE SHIFT MANAGER

The parent who is on shift must NOT wear earplugs, headphones, or use any sleep-inducing aid. They must remain fully capable of hearing and responding to the baby. The off-duty parent may use earplugs or white noise to protect their sleep opportunity — that is the point of the system.

RULE 2: BABY CONCERNS ALWAYS OVERRIDE THE ROSTER

If the baby cannot be settled, or the active caregiver is concerned about feeding, illness, injury, breathing, or any unusual behavior — the active caregiver assesses the baby and follows their existing pediatric or clinical guidance, or seeks appropriate help. Use the agreed phrase **"I need a handoff"** to coordinate partner support if needed. The roster pauses. Baby comes first.

RULE 3: THIS PRODUCT IS A COORDINATION TOOL, NOT MEDICAL ADVICE

This guide provides household scheduling models and relationship communication tools. It does not contain clinical diagnoses, pediatric medical advice, or safe-sleep certifications. For specific feeding schedules, safe-sleep arrangements, or any physical concerns about the baby, consult your pediatrician, midwife, or certified lactation consultant.

BOTH ADULTS INITIAL BELOW TO CONFIRM THEY HAVE READ AND AGREED TO THESE GUARDRAILS BEFORE SIGNING THE ROSTER.

Initial: _____ (Mother) Initial: _____ (Partner / Helper) Date: _____

● Roster Safety & Boundary Rules

Module 04 · READ

The Three Night-Shift Contract Templates

A

Postpartum Shift Agreement

The Breastfeeding-Compatible Split Shift

For couples where the baby is exclusively breastfed. The partner manages all non-feeding operations during his shift, protecting the mother's sleep opportunity blocks.

Shift 1

HANDOFF

Shift 2

Mother

Partner / Helper

Date

A

Postpartum Shift Agreement

To protect a minimum of four (4) hours of uninterrupted sleep opportunity for [Mother's Name] every night to prevent cognitive breakdown, while ensuring [Baby's Name] is safely cared for.

9:00 PM – 2:00 AM (adjust to your household —
write your actual times below)

HANDOFF

_____ PM to _____ AM

Duties

- Active Shift Manager: [Partner's Name] — must remain alert and awake while caring for the baby.
- Duties: Diapering, burping, soothing, and resettling the baby in the crib.
- Feeding Protocol: Baby is fed by the mother immediately before the shift starts. If the baby wakes during this shift:
- Partner diaper-checks and soothes the baby first.
- If the baby needs a feed, partner brings the baby to the mother in a dark room. Mother feeds without

turning on bright lights. Immediately after feeding, partner takes the baby back — diaper, burp, soothe, resettle. Mother returns to sleep immediately.

- Active Shift Manager: [Mother's Name]

BOUNDARY RULE

Duties: All feeding, diapering, and soothing duties. Partner is off-duty and sleeps uninterrupted.

Mother

Partner / Helper

Date

B

Postpartum Shift Agreement

The Alternating / Bottle-Fed Shift

Shift 1

HANDOFF

Shift 2

Mother

Partner / Helper

Date

B

Postpartum Shift Agreement

10:00 PM – 3:00 AM (adjust to your household —
write your actual times below)

HANDOFF

_____ PM to _____ AM

Duties

- Active Shift Manager: [Partner's Name]
- Active Shift Manager: [Mother's Name]
- Prep Rule: Before the first shift starts, all bottles, formula, or expressed breastmilk must be pre-measured and placed at the feeding station. No night-time hunting for supplies.

BOUNDARY RULE

Duties: Bottle-feed (formula or expressed milk), burp, diaper change, soothe, and resettle the baby. Mother is off-duty and sleeps in a separate sleep zone.

Mother

Partner / Helper

Date

C

Postpartum Shift Agreement

If You Don't Have a Partner Available (Support-Web Roster)

Shift 1

HANDOFF

Shift 2

Mother

Partner / Helper

Date

C

Postpartum Shift Agreement

To protect the mother's physical recovery and mental wellness by scheduling support-web shifts, ensuring she gets a minimum 4-hour sleep opportunity block.

_____ PM to _____ AM

HANDOFF

Shift 2

Duties

- Night Helper / Family Member / Paid Support:
[Helper's Name]
- Active Support Nights: [Days of the week — e.g.,
Monday, Wednesday, Friday]
- Shift Block: 11:00 PM – 4:00 AM (write your actual
times below)
- Active Shift Manager: [Helper's Name]
- Duties: Diaper, soothe, and bottle-feed (formula or
expressed milk). If direct breastfeeding is required,
the helper brings the baby to the mother for feeding

only, then immediately takes the baby back to burp, diaper, and resettle. Mother's role during this block: sleep.

Mother

Partner / Helper

Date

The Sleep Banker (Recovery Bank) Mechanics

What the Recovery Bank Is

The **Recovery Bank** is a fairness accounting system. It translates invisible night exhaustion into visible, tradable credits — so that exhaustion debt does not silently compound into resentment.

Without a ledger, favors disappear. One partner does an extra night and expects recognition. The other forgets it happened. Three weeks later it surfaces as a fight about something unrelated. The Recovery Bank prevents that by making every credit visible and time-bound.

The Rules of Exchange

The Currency

CREDIT TYPE	WHAT IT IS	HOW IT IS EARNED
1 Night Shift Credit (NSC)	Covering a night block of 3–5 hours outside your scheduled roster, allowing the other parent to sleep	The partner who covers the unscheduled block earns +1 NSC
1 Recovery Payback (RP)	A morning sleep-in (until 11:00 AM) or a weekend afternoon nap of 3 hours, where the other parent takes 100% responsibility for the baby and household	Redeemed by the parent who holds the credit

The Ledger Rule

All credits and debits must be written on the physical Fridge Shift Chart or in the table below. Verbal promises do not count. If it is not written, it did not happen.

The Expiration Rule

All Night Shift Credits must be redeemed as Recovery Paybacks within **seven (7) days** of being earned. Any credit not redeemed within 7 days expires automatically. This prevents historical tally-keeping ("You owe me from three weeks ago") and forces timely rest rather than compounding resentment.

The Recovery Bank Ledger

Print this table and attach it to the Fridge Shift Chart. Update it daily.

DATE EARNED	PARENT EARNING (+1)	NSC ACTIVITY	DATE REDEEMED	NET BALANCE

How to Use It — Three Scenarios

Scenario 1: Partner covers an extra block

Baby is sick on Wednesday. Partner covers mother's shift (2–7 AM) so she can sleep. Partner earns +1 NSC. Mother writes it on the ledger Wednesday. Partner redeems it as a Saturday morning sleep-in by Sunday at the latest (7-day window).

Scenario 2: Emergency override

Any night where the on-duty parent activates the emergency handoff and the off-duty parent takes over automatically earns the covering parent +1 NSC. No discussion needed — the emergency protocol itself triggers the credit.

Scenario 3: Expired credit

Partner earned an NSC on Monday. By the following Monday it has not been redeemed. It expires. This is not a punishment — it is a system boundary. The goal is rest now, not debt-tracking later.

The Seven Script Cards

These scripts are not suggestions. They are precision tools — written for the specific moment, with the specific words that do not trigger defensiveness. Do not improvise at 3 AM. Use the script.

1

SCRIPT CARD

Initiating the Shift Conversation

WHEN TO USE · During the day — never at night — when the baby is calm. To introduce the contract system without making it feel like an attack.

“I love how hard we are both working to take care of [Baby's Name]. But my current exhaustion isn't sustainable — I am hitting a wall where I cannot function safely during the day. I want us to look at this night-shift contract template together. It's not about finding fault or blaming you. It's about mapping out a roster so we both get a guaranteed sleep opportunity block. Can we look at it for 5 minutes after lunch?”

CUT ALONG DASHED LINE

2

SCRIPT CARD

Defusing "I Have Work Tomorrow"

WHEN TO USE · When your partner objects that they need sleep because of their job the next day.

“I hear you, and your job is crucial for our family. But taking care of a newborn all day is also a full-time, high-stakes job. If I collapse from exhaustion, drop the baby, or my health fails, our whole household goes down. We both have demanding jobs now — mine starts at sunrise, yours starts at office hours. We both need a minimum safety block of sleep to do our jobs safely. Let's look at the roster template, which protects your sleep for the second half of the night.”

CUT ALONG DASHED LINE

3

SCRIPT CARD

Handling "You Do It Better" (Competence Weaponization)

WHEN TO USE • When your partner claims they don't know how to soothe the baby, or that the baby only calms for you — as a way to avoid night duty.

"The baby only prefers me because I've had more hours of practice. You are just as capable of learning how to soothe [Baby's Name]. If I step in every time they cry, you will never get that practice, and I will never get sleep. I am going to put on my white noise machine. I trust you to find your own rhythm with the baby. I'm going to sleep now."

CUT ALONG DASHED LINE

4

SCRIPT CARD

The Emergency Handoff (The 2 A.M. Valve)

WHEN TO USE • When the active shift manager has reached their limit — the baby won't settle, safety is at risk, you cannot go further.

"I have reached my limit. I am holding the baby safely but I am too exhausted to continue safely. I need to activate the emergency handoff. I am putting the baby safely in the crib now and I need you to take over. I'll be back in 30 minutes."

CUT ALONG DASHED LINE

5

SCRIPT CARD

The 5-Minute Sunday Roster Review

WHEN TO USE • Every Sunday afternoon to adjust the roster for the coming week. Non-negotiable weekly ritual — 5 minutes, no longer.

"Let's do our 5-minute night review. How did last week's shifts feel? Did you get your sleep opportunity blocks? The baby is waking up at different times now — do we need to adjust our transition time? Let's write the updated hours on the chart for this week."

CUT ALONG DASHED LINE

6

SCRIPT CARD

The No-Blame Reset (After a Failed Night)

WHEN TO USE • The morning after a night where the roster collapsed, arguments happened, or someone slept through a shift.

“Last night was rough and we both lost our tempers. Let's not blame each other — exhaustion does that to everyone. The roster didn't work because [specific reason: handoff alarm didn't go off / baby was sick]. Let's adjust the system: tonight, we will [specific fix]. We reset today with a clean slate.”

CUT ALONG DASHED LINE

7

SCRIPT CARD

Defusing Extended Family / Omugwo Critique

WHEN TO USE • When mothers, mothers-in-law, or relatives criticize your partner's involvement in night care — saying it is “not a man's job” or that night duty belongs to the woman.

“Thank you for your care and concern. This is how we protect our marriage and our baby's safety. Sharing night shifts ensures [Partner's Name] builds a strong bond with the baby from the beginning, and it allows me to heal physically from childbirth. This system works for our household, and we are committed to it.”

The Fridge Shift Chart (Printable Layout)

Every Night, Just Me

NIGHT-SHIFT CHART · swiip.com/v06



WEEK OF

ROSTER TYPE (CIRCLE ONE)

☐ A

☐ B

☐ C

Shift 1 · Evening block

___ PM to ___ AM

Active Shift Manager: _____

Diaper check · Soothe · Bottle feed (if applicable)

Off-duty parent is in Do-Not-Disturb sleep zone. Active manager must remain fully alert — no sleep aids.

TRANSITION · SILENT HANDOFF · ___ AM

Shift 2 · Morning block

___ AM to ___ AM

Active Shift Manager: _____

Breastfeed / Bottle feed · Diaper check · Soothe

Off-duty parent is in Do-Not-Disturb sleep zone.

EMERGENCY HANDOFF RULE

If baby cannot be settled, or you are concerned about feeding, illness, or safety — assess baby and follow your pediatric guidance or seek appropriate help. To coordinate partner support, say: "I need a handoff."

DATE	PARENT (+1 NSC)	NSC ACTIVITY	DATE REDEEMED

Fill in one row every morning — it takes 90 seconds.

COLUMN	WHAT TO RECORD
S.O. Block (hrs)	Sleep Opportunity Block protected — the hours you were officially off-duty. Target: 4+ hours.
Actual Block (hrs)	Actual uninterrupted sleep you got within your off-duty block.
Awakenings	Total number of times the baby woke during the night.
Responder	Who responded to each awakening — Mother / Partner / Shared.
Handoff	How the shift transition went — Smooth / Delayed / Missed / Argued.
Partner Initiative	Did the partner start his shift without being asked? — Initiated / Needed nudge / Slept through.
Exhaustion (1–10)	Mother's self-rated morning exhaustion level. 1 = rested. 10 = cannot function.
Breakdown / Notes	Why the roster failed or what caused friction — e.g., Teething, Work alarm, Baby ill, Argued.

[illegible]

DAY 7 REVIEW: How is the system performing? Adjust roster times if needed before Week 2.								
What worked: _____ What broke: _____ Adjustment for Week 2: _____								
DAY 14 REVIEW: Complete your Day 14 check-in on the Swiift platform.								
S.O. blocks ≥ 4 hrs achieved (out of 14 nights): ____ / 14								
Exhaustion trend: Getting better / Staying the same / Getting worse (circle one)								
"Whose turn" arguments this week: ____								

What to Do With the Data

- Day 7 review:** Look at the Handoff column. If "Missed" or "Argued" appears more than twice, the transition time or method needs adjusting — not the whole roster.
- Day 14 review:** Compare Exhaustion scores from Days 1–3 vs Days 12–14. If exhaustion is not trending down, bring the tracker to your Day 14 check-in and review with your support person or healthcare provider.
- Day 30 check-in:** The Swiift platform will prompt you. The tracker is your evidence.

The Rescue Card (Bad Night Protocol)

Use this protocol when the baby is sick, teething, going through a growth spurt, or when the roster has collapsed completely. This is not a failure — it is a planned recovery route.

Bad Night Protocol

WHEN TONIGHT GOES WRONG

1

Stop and Assess the Baby First

Before anything else — check the baby. If the baby has a fever, is vomiting, is showing signs of unusual pain, has difficulty breathing, or you have any concern about their physical wellbeing — suspend the roster immediately and follow your pediatric guidance or contact emergency services. Nigeria: 112 United Kingdom: 999 USA / Canada: 911 The roster resumes when the baby is stable and you have appropriate support in place. The roster never takes priority over the baby's health.

2

The 3-Step Survival Routine

If the baby is assessed and safe but the night is simply impossible — teething, growth spurt, inconsolable crying — do this: The parent whose shift it currently is handles the initial response. Do not wake the off-duty parent unless you have exhausted your options and reached your limit. If the baby will not settle at all and the situation is not improving: switch to an hourly tag-team — Parent A takes the first hour, Parent B takes the second hour. One parent rests while the other is on. Do not both stay awake — maintaining one person's functional capacity is the goal. Any night where the on-duty parent activates the emergency handoff and the off-duty parent takes over automatically earns the covering parent +1 Night Shift Credit in the Recovery Bank. Write it on the ledger in the morning. No discussion needed.

3

The Morning After

Do not review, debrief, or assign blame the same morning. Both adults are likely running on near-zero. Agree on one thing only: Then rest. Debrief at the Sunday Roster Review, not at 7 AM on a broken night.

The Re-entry Protocol

When to Use This

You had a good run — maybe a week, maybe two. Then something happened: a work trip, an illness, a hard season, an argument. The roster quietly stopped. You slid back to the pattern where you do everything alone at night, and now it feels too awkward or too exhausting to restart.

This is a normal relapse point. It is not a sign the system failed. It is a sign the system needs a restart. This protocol is built for exactly this moment.

The 3-Step Re-entry

Step 1: The 10-Minute Reset Meeting

Schedule it during the day — not at night, not in the morning after a hard night. Sit down together with the Fridge Shift Chart and the tracker.

No blame. No full debrief. One agenda item only:

"We stopped. Let's restart. Which part broke?"

Step 2: Review the Breakdown Reason

Look at the tracker or think back honestly. Ask:

- Was the shift transition time unrealistic for your schedule?
- Did a work pattern or travel change make the roster unworkable?
- Did an argument about the roster make it feel easier to abandon it?
- Did the baby's sleep change and the old times no longer fit?

Identify the one real reason. Then adjust that one thing — not the whole system.

Common adjustments: - Move the transition time by 30–60 minutes to match reality - Simplify to one non-negotiable block if the full roster feels overwhelming - Add the Sunday Review back as a hard calendar commitment

Step 3: Adjust, Re-sign, and Restart the Tracker

Make the adjustment on the roster template. Re-sign the contract — both adults. Restart the 14-Day Shift Tracker from Day 1 with the new times.

You do not restart from the diagnosis phase. You already know your situation and your roster type. You restart from Day 4 (the Run phase) — pick up the system where you know it works and run it again with the adjustment in place.

The One Rule for Re-entry

**No guilt. No lengthy debrief. No "you ruined it." One adjustment.
Re-sign. Restart.**

The system is designed to be restarted. Use it.

Safety Gate — When Exhaustion Needs More Than a Night Plan

What This Page Is For

This night-shift system is designed to protect your sleep opportunity and reduce household conflict. It does that well.

What it cannot do is treat a clinical condition. Severe, sustained exhaustion can mask or worsen postpartum mental health conditions — and a better roster will not fix that. This page helps you recognise when something more is happening, and routes you to appropriate support.

The Exhaustion vs. Clinical Warning Checklist

Review this list during your Sunday Roster Review, or any time something feels different from ordinary tiredness.

If you experience two or more of the following consistently for more than two weeks, do not attempt to manage this with sleep schedules alone. Seek professional assessment.

You feel an overwhelming sense of hopelessness, emptiness, or sadness that does not lift even after a good sleep block.

You feel intense anxiety, panic, or rapid heartbeats when thinking about the baby's sleep or night-time.

You are unable to fall asleep even when the baby is sleeping and your partner is on duty.

You experience intrusive, frightening thoughts of harm coming to yourself or the baby.

You feel disconnected from your baby, or feel as though they would be better off without you.

What to Do If You Identify These Symptoms

Stop managing this alone. Route directly to one of the following:

For Partners Reading This Page

If you have noticed these signs in the mother — she cannot name them herself, or she is minimising them — you can name it. You can say:

I've noticed something feels different. Not just tired. I want us to look at the safety checklist together. I'll come with you to speak to

someone.

That is the most important shift you can make.

This product — Every Night, Just Me — is complete. You now have a written system, a fairness ledger, the words to hold it, and a clear route out if you need more than a roster. Use all of it.

End of V06 — Every Night, Just Me

13 modules | All files located in: specs/V06/

Nigeria

MANI (24/7 MENTAL HEALTH
SUPPORT)

0809 111 6264

SURPIN (SUICIDE PREVENTION)

+234 806 210 6493

EMERGENCY SERVICES

112

United Kingdom

SAMARITANS (24/7 HELPLINE)

116 123

NHS MENTAL HEALTH SERVICES

111

EMERGENCY SERVICES

999

United States & Canada

NATIONAL CRISIS LIFELINE

988

POSTPARTUM SUPPORT
INTERNATIONAL (PSI)

1-800-944-4773

EMERGENCY SERVICES

911

Authoring *Standard*

ASSET TYPE	VISUAL PATTERN USED	WHERE IT APPEARS	WHY THIS PATTERN
editorial	prose + pull-quote + numbered-reason	01, 11	Explanation/argument module — best as prose with a pull-quote and numbered reasons.
checklist	checklist / quiz (square checkbox)	02, 12	Self-assessment — square checkboxes, generous line height.
decision-tree	decision-tree (boxed nodes + connectors)	03	Branching routing — real boxes + connectors, never an ASCII diagram.
callout	callout (semantic state)	04	A non-negotiable rule or warning — the callout shows the exact semantic state.
schedule-grid	schedule-grid (proportional shift bar)	05, 08	A time-based agreement — render as a proportional shift bar, never prose.
tracker	tracker table (navy header, zebra)	06, 09	A fillable log over time — a ruled tracker table reads as writable.
script-card	script-card (numbered badge, serif quote)	07	Verbatim words to say — the serif-italic quote marks spoken language.
rescue-card	rescue-card (physical-object, red band)	10	In-the-moment crisis steps — an object you could hold, not a paragraph.

Get Through *Tonight*

A written, fair night-shift system both adults follow — so she gets a protected sleep opportunity, and nobody has to ask.

Educational, not medical advice. This content is educational. It is not a substitute for medical, clinical, or mental-health assessment. If you are concerned about your baby or yourself, seek care.

